

العمر (تاريخ الميلاد) : 22-12-1968 المهنة : بدون عمل

ختم
شعار الجمهورية

Name: امينه السيد عويضة علي
National ID: 26812220100343



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



09/20/2026

MDT FORM

Name:	امينة السيد عروضة على	MRN:	9741
Age:	58 Y	ID:	26812220100343
Menopausal Sta	post	Phone:	01155522415
NRS SCORE:		Marital Status:	widow
BMI:		Family History:	-ve
CO Morbidity:	DM	Reason Of MDT:	Dp

Clinical Data:

R

BX:	IDC GIII
ER:	8/8
PR:	0/8
HER2:	-ve
KI67:	80%

Sonomammography:

Mets Workup:

History:

Female pt 57 yrs old with Rt breast mass metastatic to bone ?? lung , DM , no HTN , condition started with bone pain 3 months ago , received medical tt and not improved , done MRI dorsal spine 19/1 : multifocal vertebral destructive neoplastic lesions , involving D5,6and 7 , most likely metastatic deposits , PET CT 28/1/2026 : Rt breast mass , with RT axillary and mediastinal LNs , widespread lytic osseous lesions , few bilat. lung nodules that are too tiny to be characterized .

Rt breast mass trucut biopsy : IDC , GIII , ER 8/8 , PR 0/8 , HER2 -ve , KI67 80%

pt received AI since (4-2026 till now)

Pet Cts (2-8-2026) :

-Regressive course of FDG avid right breast nodular lesion and ipsilateral axillary nodes.

-Morphologic progression with mild metabolic regression of FDG avid bone deposits -Non FDG avid pulmonary nodules.

Pt clinically progressed of bony aches

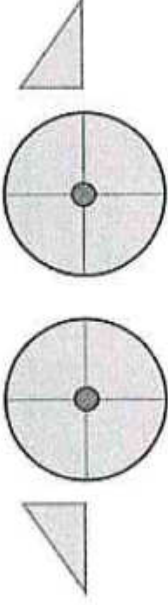
Recommendations:

after Dr khaled cons >> for MDT regarding CDK 4/6 I + faslodex

MDT Outcome & Plan

for CDK 4/6 I + faslodex
zomita every 3 ms after dental assessment

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاني عزت					

Patient's Name: Amina El-Sayed Oweida
Date: 2026-08-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: Average in size displaying homogenous parenchymal echopattern with smooth surface.

- No definite focal lesions or intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: Average distention and capacity with no evidence of stones or mud or masses inside. The wall is not thickened.

- CBD is not dilated.

Spleen: average size, homogenous.

- **Kidneys:** Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.

- No stones or dilated pelvi-calyceal system

- No gross masses or space occupying lesions could be seen.

Para-aortic and pancreatic area: masked by gases.

No ascites.

Urinary Bladder: Average capacity with no stones or diverticular out pouching

Uterus and adenexa; no abnormalities could be detected

Opinion:

Unremarkable study

Best regards

Dr. Sahl Mahmoud



Lab code 50688 Req. Date 01/08/2026

Sex Female Age 58 Years Name أمينة السيد عويضة الاستاذة

Referred by Reporting Date 01/08/2026

Kidney Function Tests

Result Reference Range

B_L urea

24.0 mg/dL 15 - 50

Serum creatinine

1.04 mg/dL 0.60 - 1.5

معمل البرود لاب
 Lab. Manager
 للتحاليل الطبية

③ ٠١٠٣٣٦٧٣١٠٠ - ٠١١٤٦٦٠٠٢٥١ - ٢٨٢٦١١٠ ت. أمام كياجي البياني ت. شقة (٥) مدخل شقة (٢) - بلك (٢) - بلك (١) - مساكن أطلس (١) آخر ش. المقطم - آخر ش. جسر السويس - خلف حديقة بدر - تقاطع ش. الأريحين مع ش. التروى ٢ عمارات سيجال الدور الأول - شقة (٣) ٠١٠١١٣٠٥٠٢٠ ت. ٠١٠٣٣٦٦٢٨١ ت. ٠١٠٣٣٦٦٢٨١ ت. أعلى حلواني كينج سويت ناصية شارع عز الدين عمر

E-mail : elrowadlabs@gmail.com

<https://www.facebook.com/elrowadlabs/>

Name: Amina Elsayed Awida Aly.
Referred by: Prof.

Date: 2-8-2026.
ID: 30209891.

F18-FDG PET/CT STUDY

Clinical History:

- 57 years old female patient with right breast cancer and multilevel vertebral destructive bony lesions received RTH and TTH.

Reason for PET/CT study:

- Assessment of response to therapy.

Technique:

- Radiopharmaceutical: 18F FDG.
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

COMPARISON: Compare to previous PET CT study dated 30-4-2026.

FINDINGS:

Head and Neck:

- No hyper-metabolic FDG avid cervical LNDs
- No FDG avid thyroid nodules
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal diffuse brain activity with NO hypermetabolic brain lesions.

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رحلة علم مدحها الحياة



اشارع سكة الإمام - السيدة زينب - القاهرة

ص. ب : الدواوين - مصر

الهاتف : 19057 (202)

فاكس : 25 35 1745 (202)

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Breasts (locoregional) & Chest:

- Morphological and metabolic regressive course as regards the previously seen right breast UIQ well defined spiculated lesion measuring currently about 2.3x1.6 cm compared to 2.7x2.6 cm in the previous study of SUVmax 1.6 (was 3).
- Rather stationary morphological regressive course as regards the previously seen enlarged right axillary LNs, the largest is measuring about 1.3x1 cm its maximum axial diameters of SUVmax 1.2 (was 3.2).
- No left breast sizable soft lesions detected.
- Rather stationary course of the previously seen bilateral scattered pulmonary nodules, the largest is seen measuring about 7 mm. NO FDG avid lung nodules or masses detected.
- Still noted enlarged retrocaval/right para tracheal LN measuring about 1x0.8 cm compared to 2x1.6 cm previously.
- Prominent right hilar LN still noted
- There is no hypermetabolic mediastinal or left axillary lymphadenopathy.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.
- Few atherosclerotic plaques of the aorta and coronaries.
- Bilateral elastofibroma dorsi.

Abdomen and Pelvis:

- There is no hypermetabolic abdominal or pelvic lymphadenopathy.
- The liver is average size and homogenous parenchymatous texture with NO FDG avid focal lesions detected.
- The spleen is of average size with NO FDG avid focal lesions detected.

- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder

Musculoskeletal:

- Overall morphological progressive course as regards the previously seen widespread heterogeneous CT texture of the examined bones with multiple lytic bony lesions, some are associated with soft tissue components the largest is seen mainly related to D6 vertebral body showing structural collapse with regressive pre and para vertebral soft tissue component measuring currently about 3.1x4.2 cm compared to 4.1x4.5 cm in the previous non-contrast study along its maximum axial diameters for MRI results correlation to delineate the extends of intra spinal extension. They show mild metabolic regression of SUVmax 3.5 (was 4.5).

IMPRESSION:

Regressive course of FDG avid right breast nodular lesion and ipsilateral axillary nodes.
 Morphologic progression with mild metabolic regression of FDG avid bone deposits.
 Non FDG avid pulmonary nodules.

Thanks for referral,

Dr/ Rehab Hussein, MD
 Radiology Consultant

Dr. Maha Mehesen, MD.
 Nuclear Medicine Consultant.

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 ص . ب : الدواوين - مصر
 الهاتف : 19057 (202)
 فاكس : 25 35 1745 (202)
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٩٧٤١

Name: Amina Elsayed Awida Aly
Referred by: Prof.

Date: 30.04.2026
ID: 30209891

F18-FDG PET /CT STUDY

Clinical History:

- 57 years old female patient with metastatic right breast Ca under treatment

Reason for PET/CT study:

- Assessment of response to therapy.

Technique:

- Radiopharmaceutical: 18F FDG.
 - Blood glucose at time of 18F-FDG injection: mg/dL.
 - PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
 - Additional notes: SUV max normalized to body weight was used.
- COMPARISON:** Compare to previous PET CT study dated 28.01.2026.

FINDINGS:

Head and Neck:

- Small subcentimetric right supra clavicular LN detected measuring about 5x8 mm , with NO pathological FDG uptake. No hyper-metabolic FDG avid cervical LNDs

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PET Scan

[[DOCTYPET.PET_SCAN]]

- No FDG avid thyroid nodules
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal diffuse brain activity with NO hypermetabolic brain lesions.

Breasts (locoregional) & Chest:

- Morphological and metabolic regressive course as regards the previously seen right breast LIQ well defined spiculated lesion measuring currently about 2.6x2.7 cm compare to 2.9x3.2 cm in the previous study with SUV max 3 (previously was 5.8).
- Morphological and metabolic regressive course as regards the previously seen enlarged right axillary LNs, the largest is measuring about 1.5x1 cm compare to 2.2x3.4 cm in the previous study along its maximum axial diameters , with SUV max 3.2 (previously was 7.3).
- No left breast sizable soft lesions detected.
- Overall mild progressive previously seen bilateral scattered pulmonary nodules, the largest is seen measuring about 7 mm compare to 5 mm in the previous study , still with NO pathological FDG uptake , for close follow-up.
- Still noted enlarged retrocaval/right para tracheal LN measuring about 2x1.6 cm in the current study compared to 1.9x1.4 cm in the previous non-contrast basis study , with SUV max 3 (previously was 4.5).
- Prominent right hilar LN at the current study measuring about 13x9 mm.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

- Few atherosclerotic plaques of the aorta and coronaries.
 - Bilateral elastofibroma dorsi.
- Abdomen and Pelvis:**
- There is no hypermetabolic abdominal or pelvic lymphadenopathy.
 - The liver is average size and homogenous parenchymatous texture with NO FDG avid focal lesions detected.
 - The spleen is of average size with NO FDG avid focal lesions detected.
 - The pancreas shows normal FDG avidity.
 - There are no active adrenal nodules.
 - Physiologic FDG excretion is seen in the kidneys and bladder

Musculoskeletal:

- Overall morphological progressive course as regards the previously seen widespread heterogeneous CT texture of the examined bones with multiple lytic bony lesions, some are associated with soft tissue components the largest is seen mainly related to D6 vertebral body showing structural collapse with almost stationary pre and para vertebral soft tissue component measuring currently about 4.1x4.5 cm compare to 4.4x4.7 cm in the previous non-contrast study along its maximum axial diameters for MRI results correlation to delineate the extends of intra spinal extension.
- However they show Mild metabolic regression with current SUV max 4.5 (previously was 6.3).

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PET Scan

[DOCTYPET SCAN]

IMPRESSION:

Morphological and metabolic Regression of the old right breast primary and metastatic right axillary and mediastinal nodal lesions.
Mild progression of the old inactive pulmonary nodules.
Mixed pattern of response of the old wide-spread predominantly lytic bone metastases with an overall Mild morphological progression.

Thanks for referral,

Aya Reda, MD

Radiology Consultant

Ahmed Zaher , MD

Nuclear Medicine Consultant

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رحلة علم هدفها الحياة



PET Scan

شارع سكة الإمام - السيدة زينب - القاهرة

ص . ب : الدواوين - مصر

الهاتف : 19057 (202)

فاكس : 25 35 1745 (202)

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[[DOCTYPET.PET_SCAN]]



Lab code 46424 Req. Date 29/04/2026
 Sex Female Age 58 Years Name الاستاذة امينة السيد عويضة
 Referred by Reporting Date 30/04/2026

Kidney Function Tests

Result Reference Range

BUN urea 23.0 mg/dL 15 - 50

Serum creatinine 0.77 mg/dL 0.60 - 1.5

Lab. Manager

فرع المقطم : آخر ش ٩ مساكن أطلس (١) - بلوك (٢) - مدخل ٢ شقة (٥) أمام كيايجي البيباني ت: ٢٨٤٧٤١١٠ - ٢٨٤٧٤١٠٠٢٥١ - ٠١١٤٦٦٠٠٢٥١
 فرع جسر السويس : خلف حديقة بدر - تقاطع ش الأربعين مع ش الترولى ٢ عمارات سيجال الدور الأول - شقة (٣) ت: ٠١١١٢٠٥٠٣٠
 فرع الهرم : ٦٥ شارع ترسا - أعلى حلواني كينج سويت ناصية شارع عز الدين عمر ت: ٠١٠٣٣٦٩٢٨١

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C) Chest:

- Few tiny bilateral lung nodules (up to 5 mm) are seen with no concordant significant increase in tracer uptake.
- enlarged mediastinal pre-tracheal LN.s (= 2 cm) is seen eliciting increased tracer uptake with **SUVmax of 3.3**.
- No other pathologic enlarged or dense tracer fixing mediastinal or hilar LN.s seen.
- No evidence pleural or pericardial effusion detected.

D) Abdomen and Pelvis:

- Average sized liver showing homogenous tracer uptake and CT texture with no focal lesions (on non-contrast CT basis), dense tracer fixing foci or biliary radicle dilatation noted.
- Normal size, parenchymal tracer uptake and CT texture of the spleen, pancreas and both adrenal glands with no definite focal lesions (on non-contrast CT basis) or dense tracer fixing foci seen.
- Normal size, parenchymal thickness and density of both kidneys showing no backpressure changes with average rate of dense tracer washout.
- Normal tracer uptake pattern and non contrast CT appearance of the pelvic organs.
- No pathologic enlarged or dense tracer fixing abdomino-pelvic LN.s seen.
- No evidence of ascites, abdominal or pelvic fluid collections noted.

E) Bony Skeleton:

- Widespread lytic osseous lesions; involving both humeral heads/proximal shafts, both scapulae, few bilateral ribs, Rt./ clavicle, sternum, different levels of spine, pelvic bones and both femoral heads/trochanteric regions; are seen eliciting increased tracer uptake with **SUVmax up to 6.2**.

OPINION:

- The study findings are of:
 - ✓ *Glucose avid Rt. breast UIQ mass; few enlarged Rt. axillary & mediastinal LN.s; in-addition to; widespread lytic osseous lesions (as above detailed); that should be regarded as being consistent with **Active Tumor Biology** and **Metastatic Rt. Breast Cancer** appears likely; however, the final judgment remains to **Biopsy** and **Histopathology** (+/- **IHC**).*
 - ✓ *Few bilateral lung nodules; that are currently **Too Tiny** to be characterized by **FDG PET**; warranting close radiological follow up.*
 - ✓ ***Free** rest of the surveyed body from any other significant glucose avid foci that would assign to any other serious pathology.*
- **Feedback data** will be truly appreciated.
- **PET/CT** therapy monitoring would be advisable.

*Thanks for your referral,
Truly yours,
Dr. Youssef Ragab Gad-Allah.
Dr. Mohamed Hassan Hassouna, MSc.
Dr. Mohamed Fat'hy Ragab, MD.*

Patient Name : الأستاذة / امينه السيد عويضة	Request No. : 13005193
Patient No. : 26812220100343	Request Date : 25/02/2026
Path No. : 534/26	Verification Date : 05/03/2026
Sex : Female	Financial Category : تأمين صحي - جوستاف روسي
Age : 57 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data

Right breast mass.BIRADs V.

Nature of specimen

Tru cut biopsy, collected on 25/2/2026, at 12:00 pm, cold ischemia time is 10 seconds.

Gross examination

Multiple (3) grayish white tissue cores ranging in size from 0.2 cm up to 1.2 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3
- * DCIS: Focal
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.

Diagnosis

Right breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade 3, with focal duct carcinoma insitu.

For further IHC (ER, PR, HER2, Ki67)

Signature

Prof. Dr. Gamal Abdel Aty
Dr.Heba Hassan Adly

Verified by :

Dr.Heba Hasan



Patient Name : الأستاذ / امينه السيد عويضة :	Request No. : 13005193
Patient No. : 26812220100343	Request Date : 25/02/2026
Path No. : 534/26	Verification Date : 05/03/2026
Sex : Female	Financial Category : جوستاف روسي - تأمين صحي
Age : 57 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.BIRADs V.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 80%.

Conclusion:

Right breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade 3, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: 80 %.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

Dr.Heba Hasan

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Page 1 of 2

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Patient Name الأستاذ / امينه السيد عويضة
Patient No. 26812220100343
Path No. 534/26
Sex Female
Age 57 Y
Referring Doc.

Request No. 13005193
Request Date 25/02/2026
Verification Date 05/03/2026
Financial Category جوستاف روسي - تأمين صحي
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 0+: as defined as incomplete membrane reaction in less than 10 % of tumor cells . IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdel Aty
Dr. Heba El-Shafeey.

Verified by :
Dr. Heba Hasan

Printing Date : 29/03/2026 09:43:01

Page 2 of 2

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NAME: امينة السيد عويضة على

Date :2026/2/9

Full field digital mammography and ultra sound on both breasts Revealed:

Clinical Summary:Right breast lump.

Technique:-

- CC and MLO view.
- High resolution Ultrasound.

Detailed findings:

Right breast

- Scattered fibroglandular pattern (ACR B)
- Ill defined speculated dense shadow seen with micro calcifications in right lower central.
- Ultrasound demonstrates
- Ill defined speculated multi lobulated heterogenous suspicious mass lesion with micro calcification and internal vascularity by Doppler study, measured (4.5x5cm) seen in lower inner Q at 5 to 6 o'clock away from nipple , 0.38 cm from skin adherent to chest wall.
- Prominent fibroglandular pattern
- Retracted nipple , Normal skin & areolar complex
- More than 3 suspicious axillary L.N seen in level 1 and 2 , oblonged shaped with muffled hilum , highly vascular, the largest seen at apex measuring 3.3x2 cm

Left breast

- Scattered fibroglandular pattern (ACR B)
- no masses, no distortion, benign vascular calcifications and rod like calcification
- Ultrasound demonstrates
- No solid or cystic masses.
- Prominent fibroglandular pattern
- Normal skin , nipples & areolar complex
- preserved axillary lymph nodes with preserved fatty hilum

Impression:

- Right breast: BIRADS V: suspicious breast mass with suspicious axillaries LN.
- Left breast: BIRADS I: normal study

Recommendation: Biopsy.

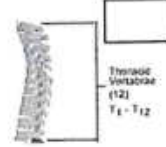




التاريخ : ٢٠٢٦/١/١٩

اسم المريض : امينه السيد عويضة

M . R . I . R E P O R T **M.R.I. Of The Dorsal Spine**



Technique:

- Sagittal T1 & T2 W Is
- Axial T1 & T2 WIs
- Sagittal T2 for whole D.S. WIs
- Post-contrast , sagittal , Axial WIs

Findings:

- Multi-focal pathological bone marrow replacing lesions are seen involving D6 , D5 and D7 (more at D6) and also D10 and to a less extent D12 and L1 vertebra , showing dark T1-, T2-signal , with post-contrast enhancement.
- Partial structural collapse of D6 vertebra and partial resorption of D10 posterior vertebral border is seen.
- Extra-osseous pathological enhancing soft tissue at the pre-vertebral space from D4/5 down to D7/8 disc level is seen . minimal intra-spinal anterior epidural enhancing tissue is also seen opposite D10 vertebra. Mild gentle indentation of the anterior aspect of spinal cord opposite D6 and D10 vertebrae , but no intra-medullary cord lesions seen.
- The dorsal discs are essentially preserved , with no posterior disc lesions.

Conclusion:

- Findings are suggestive of multi-focal vertebral destructive neoplastic lesions , most likely metastatic deposits , for clinical correlation and P.E.T. C.T. scan study.

YOURS SINCERELY,

Prof. Dr. Mohammed El-Toukhy
PROF. OF RADIO-DIAGNOSIS
CAIRO UNIVERSITY